

Class 3 – Finger Fracture: Patient Care Guide

An easy-to-understand guide for patients with a finger fracture — a break in one of the small bones (phalanges) of a finger or thumb. It covers causes, symptoms, diagnosis, treatment, healing, hand exercises, warning signs, and common questions.

Medical disclaimer: This document is an educational reference created for a patient-support knowledge base and for use in a retrieval-augmented (RAG) AI assistant. It is NOT a substitute for professional medical advice, diagnosis, or treatment. Healing times, medicines, and exercises differ for every person. Always follow the instructions of your own doctor, orthopaedic surgeon, or physiotherapist. In an emergency, contact local emergency services immediately.

1. Introduction

A finger fracture is a break in one of the small bones of a finger. Each finger has three bones called phalanges (the thumb has two), and a break can happen in any of them. It is also called a broken finger or a fractured phalanx.

Which bones are involved: the phalanges — the proximal (near the hand), middle, and distal (fingertip) phalanges. A break can also occur where the finger meets the hand bones (metacarpals). A common related injury is a mallet finger (a small break where the tendon pulls off the fingertip bone).

Common causes: jamming or crushing the finger, sports (especially ball sports), catching the finger in a door or machine, falls, and direct blows. How common it is: finger fractures are among the most common hand injuries, seen often in sports and workplaces.

Who is at higher risk: athletes in ball and contact sports, manual workers, children at play, and older adults with weaker bones. Severity ranges from a small undisplaced crack (mild) to a crushed, displaced, rotated, or open fracture (severe).

Types of Fractures Affecting This Bone

A finger fracture can happen in several patterns. Common fracture types include:

- Hairline fracture (a very thin, small crack in the bone).
- Stress fracture (a tiny crack caused by repeated force or overuse).
- Greenstick fracture (the bone bends and cracks on one side; common in children).
- Closed fracture (the bone breaks but the skin stays intact).
- Open (compound) fracture (the broken bone breaks through the skin).
- Comminuted fracture (the bone breaks into three or more pieces).
- Spiral fracture (a break that twists around the bone, often from a twisting injury).
- Impacted fracture (the broken ends are driven into each other).
- Transverse fracture (a straight break across the bone).
- Oblique fracture (a slanted or angled break across the bone).

Fingertip (distal phalanx) crush fractures, mallet finger, and small avulsion fractures (where a tendon or ligament pulls off a bone fragment) are common finger-specific patterns.

2. Anatomy

The finger is a small but complex structure of bones, tendons, and nerves.

- Bones involved: the three phalanges of each finger (two in the thumb), which form the finger's small hinge joints.
- Nearby muscles: the fingers are moved mostly by muscles in the forearm and small muscles in the hand, working through long tendons.
- Tendons: the flexor tendons (palm side) bend the finger and the extensor tendons (back) straighten it; injury can stop the finger from bending or straightening.
- Ligaments: small collateral ligaments hold the finger joints steady on each side.
- Blood vessels: tiny arteries on each side of the finger supply blood; a crushed finger can lose good blood flow.
- Nerves: small digital nerves run along each side of the finger and give feeling; injury causes numbness in the finger.

Because the finger bones are small and near the joints and tendons, even a small fracture that heals rotated or angled can cause the finger to cross over its neighbour or lose movement. Correct alignment and early, guided movement are key to a good recovery.

3. Common Causes

Most fractures happen when a force is stronger than the bone can handle. The main causes are:

- Falls: slipping, tripping, or falling onto an outstretched hand or arm — the most common cause overall.
- Road traffic accidents: high-speed impacts from car, motorbike, or cycle crashes.
- Sports injuries: contact sports, skating, cycling, gymnastics, and falls during play.
- Workplace accidents: falls from height, machinery injuries, or heavy objects.
- Direct trauma: a hard blow or hit directly on the bone.
- Twisting injuries: the limb turns while the hand or body stays fixed.
- Heavy object injuries: something heavy falling onto the limb.
- Osteoporosis: weak, thin bones that break easily, especially in older adults.
- Bone tumors: growths that weaken the bone (pathological fracture).
- Repetitive stress: repeated overuse leading to stress fractures over time.

For fingers specifically, jamming the finger (for example catching a ball), crushing it in a door, and direct blows are the most common causes.

4. Symptoms

Common signs and symptoms of a finger fracture include:

- Pain in the finger, worse with movement or pressure.
- Swelling of the finger, sometimes spreading to the hand.
- Bruising or discolouration.
- A bent, crooked, or rotated finger (deformity).

- Difficulty bending or straightening the finger.
- Numbness or tingling in the finger.
- Reduced grip or trouble holding objects.
- A dropped fingertip that will not straighten (mallet finger).
- In an open fracture, bone or a deep wound may be visible.

Emergency warning signs: a finger that is cold, blue, or white, severe crushing injury, bone through the skin, heavy bleeding, or a finger twisted out of shape. Seek urgent care to protect blood flow and function.

5. Diagnosis

Doctors confirm a fracture using a physical check and imaging tests. Each test gives different information.

Physical Examination and Medical History

The doctor asks how the injury happened and checks for pain, swelling, bruising, deformity, tenderness, movement, pulse, skin colour, and sensation. Telling the doctor about past fractures, osteoporosis, medicines, and other illnesses helps plan the right treatment.

Imaging Tests

- X-ray: the first and main test. It shows most fractures, the exact location, and whether the bone has moved out of place (displacement).
- CT scan: gives detailed 3-D images. Used for complex breaks, joint involvement, or when the X-ray is unclear, to plan surgery.
- MRI: shows soft tissues, bone bruising, and hairline or stress fractures that may not appear on an X-ray. Also checks nearby ligaments, tendons, and nerves.
- Ultrasound: sometimes used, especially in children, to look at bone surfaces and soft-tissue swelling without radiation.
- Bone scan: can detect very small stress fractures or hidden fractures that other tests miss, by showing areas of increased bone activity.

For a finger fracture, X-rays of the specific finger from more than one angle are the main test and show the fracture and any rotation or joint involvement. A CT scan is rarely needed but may help for complex joint fractures; MRI or ultrasound can assess tendon injuries such as mallet finger.

6. Treatment Options

Treatment depends on the type of break, how far the bone has moved, and your age and health. It falls into non-surgical and surgical options.

Non-Surgical Treatment

Most finger fractures are stable and heal well without surgery.

- Splint: a small finger splint holds the bone still; a mallet finger uses a special straight fingertip splint worn continuously for several weeks.
- Buddy taping: taping the injured finger to the healthy finger next to it gives support while allowing gentle movement.

- Cast: occasionally a small hand cast is used for less stable fractures.
- Brace: a hand-based brace may protect the finger in some injuries.
- Ice therapy: reduces pain and swelling in the first days.
- Elevation: keeping the hand raised above heart level reduces swelling.
- Pain medications: paracetamol for pain relief.
- Anti-inflammatory medicines: such as ibuprofen, if approved by your doctor.

When recommended: non-surgical care suits stable fractures where the bone is in a good position. Advantages: no surgery, lower infection risk, and no anaesthesia. Limitations: needs a well-set position, the cast can cause stiffness, and it may not hold badly displaced or unstable fractures.

Surgical Treatment

Surgery is used when the bone is badly out of place, broken into many pieces, involves a joint, is an open (compound) fracture, or fails to heal with a cast. The aim is to put the pieces back in the correct position and hold them still while they heal.

- ORIF (Open Reduction Internal Fixation): the surgeon opens the skin, sets the bone in the right position (reduction), and fixes it with metal implants.
- Plates: flat metal supports screwed along the bone to hold the pieces together.
- Screws: hold fragments together, alone or with a plate.
- Pins (K-wires): thin wires that hold small pieces, often used in fingers and in children; usually removed later.
- Rods (intramedullary nails): metal rods placed inside the hollow centre of a long bone to support it.
- External fixator: a frame outside the body connected to the bone by pins; used for severe, open, or badly contaminated fractures.
- Bone graft: extra bone (from your own body or a donor) added to help healing when there is bone loss or slow healing.

Indications (when surgery is needed): displaced, rotated, or unstable fractures, fractures involving the joint surface, multiple fragments (comminuted), open fractures, and some mallet or avulsion injuries. Fingers are often fixed with fine pins (K-wires) or small screws and plates.

Recovery Expectations and Risks After Surgery

- Recovery: a protective cast, splint, or sling is often used after surgery, followed by physiotherapy. Implants usually stay in place unless they cause problems.
- Risks: infection, bleeding, nerve or blood vessel injury, implant irritation or loosening, stiffness, blood clots, and reaction to anaesthesia.
- Most people regain good function, but full recovery takes weeks to months and depends on the injury and following rehabilitation.

7. Healing Timeline

Healing time varies by age, general health, fracture type, and how well instructions are followed. The table gives typical ranges.

Group	Typical healing / recovery
Children	Often 3–4 weeks to unite; small finger bones heal quickly.
Adults	About 4–6 weeks for bone union; full strength over 2–3 months.
Older adults	6 weeks or more; stiffness is the main challenge.
Athletes	Union in 4–6 weeks; return to sport often 6–12 weeks with protection.

Stages of healing: initial healing (inflammation and soft callus) in the first weeks, bone union (hard callus bridges the break), and functional recovery (regaining strength and movement). Regaining a full fist and full straightening is the key functional goal.

- Return to work: light/desk work often within days; heavy hand work usually 4–8 weeks.
- Return to sports: with buddy taping some sports return in 3–6 weeks; full contact 6–12 weeks.
- Full recovery: 2–3 months for most; swelling and stiffness can linger a little longer.

Factors that delay healing: smoking, alcohol, poor nutrition, diabetes, osteoporosis, infection, poor blood supply, older age, certain medicines (such as steroids), and not resting the injured limb.

8. Recovery Stages

Knowing what to expect at each stage helps you feel reassured. Your own timeline may differ slightly.

Stage	What usually happens
Week 1	Pain and swelling greatest; finger splinted or buddy-taped; keep hand raised.
Week 2	Swelling easing; gentle guided movement may begin if advised to prevent stiffness.
Week 3–4	Bone knitting; many splints continue; work on gentle bending and straightening.
Week 5–8	Bone usually united; focus on regaining a full fist, grip, and strength.
Month 3	Most movement and strength restored; residual swelling may remain.
Month 6	Full recovery for most; the finger joint may look slightly larger permanently.

9. Daily Care Instructions

- Keep any cast or splint dry: cover it while washing; a wet splint irritates the skin.
- Sleeping position: rest the hand on a pillow, raised, to reduce night-time swelling.
- Bathing: use a waterproof cover or wash carefully; dry the splint area well.
- Clothing: choose loose sleeves and avoid tight rings (remove rings early, as swelling can trap them).
- Walking: unaffected — normal walking is fine.
- Driving: avoid driving until you can grip the wheel safely and painlessly.
- Returning to office: light desk and computer work is often possible within a few days.
- Returning to school: children can usually attend, avoiding sports and rough play.

- Household activities: avoid gripping, lifting, and tasks that stress the finger.

10. Pain Management

Some pain is normal after a fracture, especially in the first days. These safe, simple methods help control it.

- Ice packs: wrap ice in a cloth and apply for 15–20 minutes several times a day to reduce pain and swelling (never put ice directly on the skin).
- Elevation: keep the injured limb raised above the level of the heart to reduce throbbing and swelling.
- Rest: avoid using the injured limb for gripping, lifting, or bearing weight.
- Painkillers: paracetamol (acetaminophen) is commonly used. Anti-inflammatory medicines such as ibuprofen may be advised. Take only the dose your doctor approves.
- Safe positioning: support the limb on a pillow and avoid letting it hang down.
- Relaxation techniques: slow breathing, distraction, and gentle music can reduce the sensation of pain.

When pain is abnormal: pain that suddenly becomes severe, is not relieved by medicine, comes with numbness, a cold or blue hand, or a very tight cast may signal a serious problem such as compartment syndrome or poor blood supply. Seek medical help urgently.

11. Diet for Bone Healing

Good nutrition speeds up bone healing. The nutrients below all play a role. Try to eat a balanced diet that includes them every day, and follow any special diet advice from your doctor or dietitian.

Key Nutrients for Bone Healing

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Calcium	Main mineral that builds and repairs bone. About 1000–1200 mg/day for adults.	Milk, curd, paneer, cheese, ragi, tofu, green leafy vegetables, sesame seeds	Small fish eaten with bones (sardines), dairy
Vitamin D	Helps the body absorb calcium. About 600–800 IU/day (more if deficient).	Sunlight, fortified milk and cereals, mushrooms	Egg yolk, fatty fish (salmon, mackerel), fish liver oil
Protein	Builds the collagen framework of bone. About 1.0–1.5 g per kg body weight.	Dal, beans, chickpeas, rajma, soya, paneer, tofu, nuts	Eggs, chicken, fish, lean meat
Vitamin C	Needed to make collagen for bone repair. About 65–90 mg/day.	Oranges, lemon, amla, guava, capsicum, tomato, broccoli	(Mainly from plant foods)
Vitamin K	Helps bind calcium into bone. About 90–120 mcg/day.	Spinach, kale, broccoli, cabbage, green leafy vegetables	Eggs, some dairy

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Magnesium	Supports bone structure and vitamin D use. About 310–420 mg/day.	Nuts, seeds, whole grains, bananas, dark leafy greens	Fish
Phosphorus	Works with calcium to harden bone. About 700 mg/day.	Dairy, beans, lentils, whole grains, nuts	Meat, fish, poultry, eggs
Zinc	Helps bone cells grow and repair. About 8–11 mg/day.	Whole grains, legumes, seeds, nuts	Meat, shellfish, eggs
Omega-3 fats	Reduce inflammation and support healing.	Flaxseed, chia seeds, walnuts, soybean oil	Fatty fish (salmon, mackerel, sardines)
Collagen	Forms the flexible base of bone. Supported by protein + vitamin C.	Body makes it from protein-rich plant foods + vitamin C	Bone broth, chicken, fish, eggs

Drink enough water and, if your doctor recommends, take calcium and vitamin D supplements. Do not take high-dose supplements on your own without medical advice.

12. Foods to Avoid

Some foods and habits slow bone healing by reducing calcium absorption, increasing inflammation, or harming blood supply. Limit or avoid the following while your fracture heals.

- Alcohol: slows new bone formation and reduces calcium and vitamin D use. Best avoided completely during healing.
- Smoking and tobacco: reduce blood flow to the bone and are a leading cause of delayed healing and nonunion. Stopping greatly improves healing.
- Sugary foods: too much sugar can increase inflammation and displace healthy foods.
- Soft drinks and colas: high phosphoric acid and sugar may reduce calcium balance.
- Excess caffeine: large amounts of coffee, tea, or energy drinks can reduce calcium absorption; keep intake moderate.
- Processed and packaged food: often high in salt and low in nutrients.
- High sodium (salt) intake: makes the body lose more calcium in urine.
- Junk and deep-fried food: high in unhealthy fat and low in bone-building nutrients.

In short, a fresh, home-cooked, balanced diet with less salt, sugar, alcohol, and no smoking gives your bone the best chance to heal quickly.

13. Physical Therapy and Rehabilitation Exercises

Physiotherapy restores movement, strength, and function. Start only when your doctor allows, and stop any exercise that causes sharp or increasing pain. Below are typical stages — your therapist will tailor them to you.

Early Stage

- Gentle movement of the uninjured fingers, thumb, wrist, and elbow — purpose: keep the rest of the hand mobile. Repetitions: 10 each, several times daily.
- Elevation and light finger wiggling (if allowed) — purpose: reduce swelling and stiffness. Precaution: keep a mallet-finger splint on at all times.

Middle Stage

- Active finger bending toward the palm — purpose: regain a fist. Repetitions: 10 slow bends, several times daily. Precaution: no forcing painful movement.
- Finger straightening exercises — purpose: restore full extension.
- Buddy-taped movement — purpose: safe motion while protecting the healing bone.

Advanced Stage

- Grip and pinch strengthening with putty — purpose: rebuild hand strength. Repetitions: 10–15 squeezes, 2–3 sets. Precaution: start soft.
- Full-fist and finger-spread exercises — purpose: restore complete movement.
- Return to fine tasks and sport-specific drills — purpose: full function. Precaution: protect the finger with tape early on.

General exercise precautions: move slowly, never push through sharp pain, stop if you feel numbness or dizziness, and follow the exact repetitions your therapist advises. When to stop: stop and call your therapist if pain worsens, swelling increases, or movement suddenly decreases.

14. Activities to Avoid During Healing

Avoiding certain activities protects the healing bone and implants.

Activity	How long to avoid (typical)
Gripping and squeezing hard	Until healed — often 4–6 weeks
Ball sports without buddy taping	About 6 weeks (then tape for protection)
Heavy lifting / carrying by that hand	About 6–8 weeks or until cleared
Contact sports (cricket, football)	About 6–12 weeks
Weight training that loads the fingers	About 8 weeks or until cleared
Removing a mallet-finger splint early	For the full 6–8 weeks prescribed

15. Possible Complications

Most fractures heal well, but knowing the possible problems helps you spot them early. Report any of these to your doctor.

- Delayed union: the bone heals more slowly than expected. Prevented by good nutrition, not smoking, and following restrictions.
- Nonunion: the bone fails to join. Signs are ongoing pain and movement at the break site; may need surgery or a bone graft.

- Malunion: the bone heals in the wrong position, causing deformity or reduced function; sometimes needs correction.
- Infection: mainly with open fractures or after surgery. Signs are fever, redness, warmth, swelling, pus, or bad smell.
- Stiffness: nearby joints become stiff from being kept still; prevented by physiotherapy and gentle movement as advised.
- Nerve injury: causes numbness, tingling, or weakness; often improves but should be checked.
- Blood vessel injury: rare but serious; causes a cold, pale, or blue limb and needs emergency care.
- Chronic (long-lasting) pain: pain that continues after healing; managed with therapy and pain specialists.
- Arthritis: joint fractures can lead to wear-and-tear arthritis years later.
- Reduced mobility: lasting loss of full movement, reduced by good rehabilitation.
- Complex Regional Pain Syndrome (CRPS): an uncommon condition with severe pain, swelling, skin colour and temperature changes; early treatment gives the best result.

16. Red Flag Symptoms — Go to Hospital Immediately

Get emergency medical help right away if you notice any of the following. These can mean the blood supply, nerves, or the healing is in danger.

- Severe or rapidly increasing swelling.
- Fingers (or toes) turning blue, grey, or very pale.
- The hand or limb feels cold to touch.
- Numbness or loss of feeling that does not go away.
- Fever or chills (a sign of infection).
- Bleeding that will not stop.
- Severe pain that is not relieved by rest, elevation, or prescribed medicine.
- The cast becomes too tight or the limb feels squeezed.
- The cast breaks, cracks, or becomes loose.
- Pus, discharge, or fluid coming from under the cast or a wound.
- A bad smell coming from the cast or wound.
- Bone is visible through the skin (open fracture).
- Difficulty breathing, chest pain, or coughing after the injury (a possible blood clot or fat embolism — a rare but serious emergency).

Compartment syndrome warning: severe deep pain, tightness, numbness, and pain when the fingers are gently stretched can signal dangerous pressure build-up. This is an emergency — do not wait.

17. Follow-Up Schedule

Regular follow-up lets the doctor check that the bone is healing in the right position. A typical schedule is below.

Visit	What the doctor usually checks
Week 1	Check swelling, splint fit, rotation, blood flow, and feeling; X-ray if needed.
Week 2	Confirm the bone has not shifted; begin gentle movement if advised.
Week 4	X-ray for early healing; progress movement; check for stiffness.
Week 6	Check union; remove pins if used; advance strengthening.
Week 12	Confirm full recovery of movement and strength; clear for full activity.

18. Returning to Normal Life

These are general guides for when people usually return to activities. Always get your doctor's clearance first.

Activity	Typical time to return
School	A few days (no sport/rough play)
Office (desk job)	A few days to 1 week
Driving	Once a safe, painless grip returns — often 2–4 weeks
Gym (upper body)	About 6–8 weeks, avoiding heavy grip early
Swimming	After splint off and skin healed — about 4–6 weeks
Cycling	Light cycling around 4–6 weeks (safe grip needed)
Running	Usually within 1–2 weeks (does not stress the finger)
Cricket / Football	About 6–12 weeks, often with buddy taping
Weight lifting	About 8 weeks, increasing grip load slowly

19. Prevention of Future Fractures

After one fracture, it is worth protecting your bones for the future. These steps lower the chance of breaking a bone again.

- Bone-strengthening exercises: regular resistance and muscle-strengthening activity keeps bones dense.
- Weight-bearing exercise: walking, jogging, dancing, and stair climbing build bone strength.
- Balance training: yoga, tai chi, and balance drills reduce falls, especially in older adults.
- Vitamin D: from sunlight, diet, or supplements as advised, to help absorb calcium.
- Calcium intake: eat enough calcium-rich foods every day.
- Protective equipment: wrist guards, elbow pads, and helmets during sports and skating.
- Safe workplace habits: follow safety rules, use guards, and prevent falls from height.
- Sports safety: warm up, use correct technique, and wear the right gear.
- Fall prevention: good footwear, adequate lighting, and vision checks.
- Home safety tips: remove loose rugs, add grab bars, keep floors dry and clutter-free.

- Osteoporosis screening: a bone density (DEXA) scan for older adults and high-risk people, with treatment if bones are weak.

20. Frequently Asked Questions (FAQ)

Q1. Can I move my finger?

Gentle movement of nearby fingers, the thumb, and wrist is encouraged. The injured finger is moved only when your doctor allows, to avoid stiffness without disturbing the healing bone.

Q2. Can I sleep on the injured hand?

Avoid pressing on the injured finger. Rest the hand on a pillow, raised, to reduce swelling and pain.

Q3. Can I remove the splint?

No, keep the splint on as instructed. A mallet-finger splint especially must stay on continuously for the full period, or healing restarts.

Q4. When can I drive?

Once you can grip the wheel safely and without pain — often around 2–4 weeks. Confirm with your doctor.

Q5. Can I exercise?

Yes, lower-body and cardio exercise is usually fine. Avoid gripping and hand-loading exercises until cleared.

Q6. Can I lift weights?

Not until the bone heals and grip is pain-free — usually about 8 weeks, starting light.

Q7. Is swelling normal?

Yes, finger swelling is common for several weeks. Keep the hand raised and use ice. Report severe or increasing swelling or a cold, blue finger urgently.

Q8. Is itching inside the splint normal?

Mild itching is common. Do not poke objects inside; cool air can help. Tell your doctor if the skin becomes sore or smells.

Q9. Can I travel?

Yes, once pain is controlled. Keep the hand raised on long journeys to limit swelling.

Q10. Can I fly?

Air travel is usually fine; tell your doctor if it is very soon after the injury, as swelling can increase.

Q11. Can I get the splint or cast wet?

Keep a plaster splint or cast dry. Some removable splints can be taken off briefly to wash — only if your doctor says so.

Q12. How long should I wear the splint?

Simple fractures often need 3–4 weeks; a mallet finger needs about 6–8 weeks of continuous splinting.

Q13. When should I see a doctor urgently?

For a cold, blue, or white finger, severe pain, spreading numbness, heavy bleeding, or signs of infection (pus, fever, bad smell).

Q14. Why does my finger look crooked?

Swelling and the fracture can make it look bent. If it is truly rotated or crossing another finger, it must be corrected — see your doctor.

Q15. Will my finger be stiff afterwards?

Some stiffness is common. Guided movement and hand exercises restore most or all motion over a few weeks to months.

Q16. Will I need surgery?

Most finger fractures do not. Surgery is used for displaced, rotated, joint, or open fractures, often with fine pins or small screws.

Q17. Do the pins need to be removed?

Small pins (K-wires) are often removed in the clinic after a few weeks. Small screws or plates usually stay unless they cause problems.

Q18. My fingertip won't straighten — what is that?

That is likely a mallet finger, where the extensor tendon pulls off a bone fragment. It is treated with a special splint held straight for several weeks.

Q19. Is a little pain normal weeks later?

Mild aching during healing and exercises is normal. Sharp or worsening pain should be checked.

Q20. What foods help my finger heal?

Calcium, vitamin D, protein, and vitamin C support bone repair. Avoid smoking and alcohol, which slow healing.

Q21. Can I go to work?

Light desk work is often possible within days. Heavy hand work usually needs 4–8 weeks.

Q22. Can I type or use my phone?

Light typing and texting with the other fingers is usually fine if it does not hurt the injured finger.

Q23. Can I play sports again?

Yes. With buddy taping some sports resume in 3–6 weeks; full contact play often 6–12 weeks, after clearance.

Q24. Will my knuckle stay swollen or larger?

The joint may look slightly larger permanently after some fractures, but function is usually good.

Q25. How can I prevent another finger fracture?

Use protective taping or gloves in sport, keep bones strong with good nutrition, and take care around doors and machinery.

Q26. Should I remove my rings?

Yes, remove rings early. Swelling can trap a ring and cut off blood flow to the finger.

21. Metadata (for Retrieval)

Structured tags to help the AI assistant retrieve this document accurately.

Field	Value
Fracture Name	Finger Fracture
Bones Involved	Phalanges (proximal, middle, distal); sometimes metacarpal border
Body Region	Hand (finger/thumb)
Common Causes	Jamming, crushing, ball sports, doors/machinery, falls, direct blows
Symptoms	Pain, swelling, bruising, crooked/rotated finger, stiffness, numbness, dropped fingertip (mallet)
Treatment Options	Splint, buddy taping, cast; pins (K-wires), small screws/plates for unstable or joint fractures
Healing Duration	Children 3–4 weeks; adults 4–6 weeks; full recovery 2–3 months
Rehabilitation Duration	3–8 weeks of hand exercises; stiffness may take longer
Emergency Signs	Cold/blue/white finger, crush injury, open bone, heavy bleeding, infection
Keywords	finger fracture, broken finger, fractured phalanx, mallet finger, jammed finger, avulsion fracture
Synonyms	Broken finger, cracked finger bone, fractured phalanx, hand fracture
Related Injuries	Wrist fracture, metacarpal (hand) fracture, tendon injury, dislocated finger